

Acute pain

Revised: August 26, 2026

Goals

- The patient will identify factors that relieve pain.
- The patient will identify an acceptable level of pain.
- The patient will verbalize or demonstrate pain control.
- Other:

Interventions

- Assess the patient's pain (characteristics, severity, location, onset, type, precipitating factors, and duration), using an age-appropriate, facility-approved pain scale.
- Identify the patient's acceptable level of pain.
- Observe the patient for nonverbal cues of discomfort, such as restlessness, muscle tension, or altered vital signs.
- Acknowledge the patient's pain, and accept the patient's response to pain.
- Assess the patient's knowledge and beliefs about pain.
- Explore factors that trigger, worsen, or relieve the patient's pain.
- Collaborate with the patient about possible [pain management](#) strategies.
- Assist the patient into a comfortable position.
- Administer prescribed medications. Monitor their effect.
- Teach the patient and family or caregiver (as appropriate) about the medication, including the medication's name, dose, frequency, route, duration, possible side effects, and interactions with other drugs, over-the-counter medications, vitamins, herbal supplements, or food.
- Treat the patient's pain as ordered, using pharmacologic, nonpharmacologic, or a combination of approaches.
- Explore nonpharmacologic [pain management](#) strategies. These include massage therapy, relaxation therapy, diversional activities, and guided imagery. Allow culturally relevant nonpharmacological strategies, as appropriate.
- Modify [pain management](#) measures based on the patient's response.
- Encourage the patient to participate in distraction activities.
- Dim the lights to provide a calming environment.
- Teach the patient and family or caregiver (as appropriate) about what causes the pain and its likely duration.
- Teach the patient and family or caregiver (as appropriate) about ways to decrease factors that worsen pain.
- Instruct the patient and family or caregiver (as appropriate) to notify the practitioner if pain control measures are inadequate.
- Collaborate with a [pain management](#) specialist, if appropriate.
- Other:

Additional Reading

1. Al Hussainy, H. A., et al. (2024). Pain score levels acceptable to patients and hospital staff in trauma and orthopaedics in a large district general hospital. *Journal of Surgery and Anesthesia Research*, 5(1), 1–5. (Level IV)
2. Gulanick, M., & Myers, J. L. (Eds.). (2022). *Nursing care plans: Diagnoses, interventions, and outcomes* (10th ed.). Elsevier.
3. Hosseini, F., et al. (2022). Acute pain management for older adults. *Clinical Medicine*, 22(4), 302–306. Retrieved July 2026 from <https://doi.org/10.7861/clinmed.22.4.ac-p>
4. Ling, J. F. J., et al. (2025). 65 empowering patients and nurses in acute pain management. *BMJ Open Quality*, 14. Retrieved July 2026 from <https://doi.org/10.1136/bmjog-2025-QSHU.65> (Level VI)
5. Magerman, Y. N., et al. (2025). Best-practice guidelines for nurses for the assessment and management of acute trauma-related pain in adult patients: A scoping review. *International Journal of Africa Nursing Sciences*, 23, 100888. Retrieved July 2026 from <https://doi.org/10.1016/j.ijans.2025.100888> (Level I)
6. Mariano, E. R. (2026). Approach to the management of acute pain in adults. In: *UpToDate*, Maniker, R. (Ed.).
7. Pandharipande, P., & Hayhurst, C. J. (2025). Pain control in the critically ill adult patient. In: *UpToDate*, Parsons, P. E., & O'Connor, M. F. (Eds.).